

THE GLP-1 LIFESTYLE SERIES

The GLP-1 Nutrition Guide

*What to Eat to Preserve Muscle and
Maximize Results*

Disclaimer

This book is for educational and informational purposes only. It is not medical advice and does not constitute a recommendation to start, stop, or change any medication or treatment. GLP-1 receptor agonists are prescription medications that should only be used under the supervision of a qualified healthcare provider.

The nutritional guidance, exercise information, and lifestyle strategies in this book are general wellness information and are not tailored to your individual medical situation. Always consult your doctor, registered dietitian, or other qualified health professional before making changes to your diet, exercise routine, or medication management.

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How GLP-1 Medications Change Your Relationship with Food

GLP-1 receptor agonists — Ozempic, Wegovy, Mounjaro, Rybelsus — work partly by slowing gastric emptying, reducing appetite, and creating a sustained sense of fullness. This is useful for weight loss. It also creates specific nutritional challenges that nobody warns you about, and that are responsible for a lot of the side effects and suboptimal results people experience.

What GLP-1 Drugs Actually Do

GLP-1 (glucagon-like peptide-1) is a naturally occurring gut hormone released in response to food intake. It stimulates insulin secretion, suppresses glucagon, slows the rate at which the stomach empties, and signals satiety to the brain. GLP-1 receptor agonists mimic and amplify this effect pharmacologically.

The result: food moves through the stomach more slowly, appetite is significantly reduced, and the brain receives earlier and stronger satiety signals. For many people, the reduction in appetite is profound — foods

that used to be difficult to resist hold little appeal, and hunger itself may feel muted or absent.

This appetite suppression is the mechanism of weight loss. It's also why eating enough of the right things becomes an active challenge rather than a passive activity. When you're not hungry, you may not eat enough protein, enough micronutrients, or enough food overall to support lean mass preservation — which is the central nutritional challenge on GLP-1.

The Muscle Loss Problem

Weight loss on GLP-1 medications follows the same pattern as all calorie-restricted weight loss: it includes both fat and lean mass. Studies on semaglutide (the active ingredient in Ozempic and Wegovy) have shown that approximately 25–40% of weight lost can be lean mass, including muscle.

This matters for several reasons. Muscle is metabolically active — it burns calories at rest and supports healthy blood sugar regulation. Losing significant lean mass during weight loss lowers metabolic rate, increases the risk of weight regain, and in older adults, contributes to sarcopenia — the age-related loss of muscle that is associated with falls, frailty, and poor health outcomes.

The nutrition and exercise strategies in this book are specifically designed to shift that ratio — to maximise fat

loss and minimise lean mass loss while on GLP-1 medication.

"GLP-1 medication handles the appetite. Your job is to make sure what you do eat is working hard enough to protect your muscle."

The Protein Priority

If there is one nutritional principle that matters above all others on GLP-1, it's this: eat protein first, at every meal, every day.

Why Protein Is Non-Negotiable

Protein is the primary dietary determinant of lean mass preservation during calorie restriction. The research is consistent: higher protein intakes during weight loss preserve more muscle. The threshold that most researchers identify for muscle preservation is 1.2–1.6 grams of protein per kilogram of body weight per day — higher than typical dietary guidelines, which are based on minimum requirements rather than optimal body composition outcomes.

On GLP-1, when total food intake is significantly reduced, getting this amount of protein requires deliberate planning. If you're eating 1,200–1,500 calories per day and protein needs to be 120–160 grams, protein needs to constitute 40–50% of total calories. This leaves limited room for other macronutrients, which is why the composition of each meal matters so much.

Best Protein Sources on GLP-1

The most practical protein sources for GLP-1 users are complete animal proteins with high leucine content. Leucine is the specific amino acid that most directly stimulates muscle protein synthesis, and it's most concentrated in animal proteins. Eggs (particularly egg whites for dense protein with low volume), chicken and turkey breast, Greek yoghurt, cottage cheese, fish and seafood, and lean beef are all excellent choices.

For those eating less meat, a combination of dairy proteins (whey, casein) with plant proteins (soy, pea) can reach complete amino acid profiles. Soy is the most complete plant protein — tempeh and edamame are high-leucine soy sources. Combining with a high-quality protein powder (whey or pea-based) is often practical when appetite is suppressed and food volume is limited.

Protein shakes deserve specific mention: when solid food is unappealing due to GLP-1 side effects, a protein shake provides muscle-preserving protein in a tolerable liquid form. A 25–30 gram protein shake consumed in the morning can anchor the day's protein intake when breakfast is the most difficult meal.

The Eat-Protein-First Rule

At every meal, eat protein before anything else. This is practical in two ways: it ensures protein is consumed even when appetite fails before the meal is finished, and

it takes advantage of the early satiety signal by filling the available appetite with the most important macronutrient first.

A plate structured as protein first, then vegetables, then any carbohydrates — rather than eating from all parts of the plate simultaneously — consistently improves protein intake in GLP-1 users without requiring larger total meal volumes.

"On a reduced appetite, every bite has to earn its place. Protein earns its place first."

Managing Nausea and Tolerating Food

Nausea is the most common side effect of GLP-1 medications, particularly in the first weeks and during dose escalations. This chapter is about eating through it without abandoning nutrition.

Why GLP-1 Causes Nausea

The nausea of GLP-1 medications is primarily a consequence of delayed gastric emptying. Food sits in the stomach longer than usual, and the pressure and fermentation that results produces nausea, discomfort, and sometimes vomiting. It tends to be worst in the first two to four weeks of a new dose and typically improves as the body adapts.

Understanding the mechanism helps identify solutions. Anything that further slows gastric emptying — large meals, high-fat foods, carbonated beverages, lying down after eating — worsens nausea. Anything that allows the stomach to process food without additional mechanical stress — small portions, upright posture after eating, liquid nutrition — reduces it.

The Nausea Protocol

Eat small, frequent portions rather than structured meals when nausea is significant. Four to six small protein-forward servings throughout the day manage nausea better than three meals. Portion size matters: when the stomach is already processing slowly, adding more volume creates pressure. Aim for portions that feel comfortable, not complete.

Cold or room-temperature foods are generally better tolerated than hot foods during high-nausea periods — heat exacerbates the stomach discomfort. Ginger — in tea, capsule form, or as a food — has genuine evidence for reducing nausea and is safe to use alongside GLP-1 medications.

Stay upright for at least thirty to forty-five minutes after eating. Gravity assists gastric emptying and reduces the pressure that produces nausea. Lying down immediately after eating is one of the most reliable ways to worsen GLP-1 nausea.

Foods That Work When Nothing Sounds Good

In high-nausea periods, the goal is maintenance — keeping enough protein and hydration in to prevent muscle loss and functional decline — rather than

optimisation. Greek yoghurt, cottage cheese, protein shakes, scrambled eggs, and soft fish are typically the best-tolerated protein sources when nausea is significant.

Saltine crackers and white rice are often recommended during nausea because they're easy on the stomach. They are, however, almost entirely void of protein. If you're relying on these for tolerance, pair them with a protein source — even a tablespoon of peanut butter on crackers is better than crackers alone.

"Getting through the nausea phase is a short-term challenge. The decisions you make during it set the foundation for everything that follows."

Hydration and Micronutrients

Reduced food intake on GLP-1 creates micronutrient gaps that are easy to miss because the symptoms are gradual and non-specific.

Hydration on GLP-1

Reduced food intake means reduced water intake from food – roughly 20% of daily water typically comes from food, not drinks. On GLP-1, when food intake drops significantly, deliberate hydration becomes more important. Dehydration amplifies nausea, fatigue, and headaches – the same symptoms already produced by the medication.

A practical target: at least 2–2.5 litres of water per day, consumed steadily throughout the day rather than in large amounts at once. Large amounts of liquid at once distend the stomach and worsen nausea. Small, frequent sips are better tolerated.

Electrolytes deserve attention, particularly if nausea has led to vomiting or significantly reduced food intake. Sodium, potassium, and magnesium – the primary electrolytes lost in reduced food and fluid scenarios –

are worth supplementing during high-symptom periods. An electrolyte powder without sugar added to water is practical and effective.

The Micronutrient Gap

When calorie intake drops significantly, micronutrient intake drops proportionally unless deliberate effort is made to include nutrient-dense foods. The nutrients most commonly insufficient on GLP-1 — based on dietary analysis studies and clinical experience — are: B12, iron (particularly in women), zinc, folate, calcium, and vitamin D.

A comprehensive daily multivitamin is the minimum appropriate supplement on GLP-1. It doesn't replace food micronutrients, but it provides a baseline that reduces the risk of significant deficiency. A bariatric-formulated multivitamin is a good option — these are designed for people eating significantly less than typical and have higher bioavailability forms of key nutrients.

"You're eating less. That means every food choice has to carry more nutritional weight."

Long-Term Nutrition Strategy

GLP-1 medications work best as part of a long-term lifestyle change, not as a temporary intervention. The nutrition strategy that serves you during active weight loss is different from the one that serves you in maintenance.

Transitioning to Maintenance

As weight loss slows and body composition stabilises, the nutrition priority shifts from aggressive protein-first eating to a more balanced, sustainable pattern. Calorie needs increase slightly as body weight stabilises, and the appetite suppression of GLP-1 may need to be managed upward — ensuring you're eating enough to support activity, recovery, and metabolic health.

Maintenance on GLP-1 looks like: adequate protein (1.0–1.2g/kg body weight), abundant vegetables, appropriate carbohydrate around activity, and quality fats. It looks, in other words, like the Mediterranean diet pattern — which consistently shows the best long-term outcomes for cardiovascular health, metabolic health, and body composition maintenance.

What to Do If the Medication Stops

The weight regain pattern after stopping GLP-1 medications is well-documented. Without the pharmacological appetite suppression, hunger returns – often to levels higher than before treatment, due to the body's homeostatic response to weight loss. Without behavioural and dietary foundations built during treatment, most of the weight is regained within one to two years.

The most protective factors against rebound are: the strength training habits built during treatment (which maintain a higher metabolic rate), the eating patterns established (particularly protein-first, vegetable-rich meals), and the relationship with hunger that was developed – learning to eat to nourishment rather than to appetite.

"GLP-1 gives you a window. What you build inside that window determines whether the results last."
